INDICATION FOR EXAM: A 59-year-old male presenting for routine colorectal cancer surveillance following a prior history of adenomatous polyps five years ago. Patient reports occasional intermittent hematochezia.

BOWEL PREPARATION: Split-dose polyethylene glycol electrolyte solution; Boston Bowel Preparation Scale score was 9 (Right colon: 3, Transverse colon: 3, Left colon: 3), indicating excellent mucosal visibility.

ENDOSCOPIC FINDINGS: The adult Olympus PCF-H190 colonoscope was introduced through the anal verge under intravenous propofol sedation administered by anesthesiology. Digital rectal examination prior to insertion was unremarkable with normal sphincter tone and no palpable rectal masses. The instrument was smoothly advanced through the rectum, sigmoid colon, descending colon, splenic flexure, transverse colon, hepatic flexure, and ascending colon to the cecum. Cecal intubation was definitively confirmed by identification of the appendiceal orifice, the triradiate cecal fold, and the ileocecal valve. Total insertion time was 6 minutes. The terminal ileum was intubated through the ileocecal valve for 15 cm and displayed normal villous architecture without ulceration or erythema.

Upon meticulous withdrawal, total mucosal inspection time was 11 minutes:

1. Ascending Colon: A 12 mm flat, Paris classification IIa sessile serrated lesion was detected at the hepatic flexure. The lesion was lifted with a submucosal injection of 4 mL saline containing 0.01% methylene blue, creating a distinct polyp bleb. The lesion was cleanly resected en bloc using an Olympus 15 mm round stiff wire cold snare without immediate bleeding or perforation. The specimen was retrieved via suction trap for pathology.

2. Transverse Colon: A 6 mm pedunculated (Paris Ip) polyp was identified in the mid-transverse colon. Cold snare resection was executed across the stalk with complete resection.

3. Sigmoid Colon: Moderate, uncomplicated diverticulosis without signs of acute diverticulitis.

4. Rectum: Mild internal hemorrhoidal disease without active hemorrhage.

RECOMMENDATIONS:

1. Await histopathology results to determine whether surveillance interval should be 3 or 5 years.

2. Resume regular diet and routine physical activity today.

3. Contact endoscopy center immediately if fever, hematochezia, or severe abdominal pain develops.